



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Guideline

Sexual Assault Management

Reference #: FSH-ED-GUI-0005

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital, Rottnest Island Nursing Post	Hospital Wide	Medical, Nursing, Midwifery, Aboriginal Health Practitioners, Allied Health

1. Introduction

Patients may present directly to the Fiona Stanley Hospital (FSH) Emergency Department (ED) following a reported sexual assault or be referred to ED from another clinical area. This guideline outlines the management of patients who have been sexually assaulted including presentations to FSH ED, Rottnest Island Nursing Post and disclosure in other clinical areas.

Children under 13 years of age are to be referred to Perth Children's Hospital for management.

2. Terminology

Sexual Assault	Any sexual behaviour or act which is threatening, violent, forced, coercive, or exploitive and to which the person has not given consent or was not able to give consent.
First response pack	A pre-prepared envelope containing a checklist for history taking / management and a patient envelope containing important information to be given to the patient.
Early Evidence Kit (EEK)	A pre-prepared kit supplying all the equipment and instructions required to collect evidence from the patient. The kit standardises the specimens collected as well as the techniques utilised during the collection.

Forensic toxicology kit	A pre-prepared kit supplying all the equipment and instructions required to collect toxicological evidence from the patient. The kit standardises the specimens collected as well as the techniques utilised during the collection.
Trauma Informed	Services that are alert to the possibility of the existence of trauma in the lives of all consumers/patients irrespective of whether it is known to exist in individual cases. The key principles of trauma informed care include safety, trustworthiness, choice, collaboration and empowerment.
Sexual Assault Resource Centre (SARC)	A 24-hour emergency service based in an outpatient clinic environment. Provides medical care, forensic examination and counselling support to people over the age of 13 years who have been sexually assaulted within the previous 14 days. Free service for people of all sexualities and gender identities, aged 13 years and above, providing: • Crisis services, including medical care, forensic collection and psychological support, to people who have experienced a recent sexual assault (rape) in the past two weeks; and • Counselling for sexual assault/abuse experienced either recently or in the past SARC services are part of the Western Australian Department of Health based at King Edward Memorial
Non-Fatal Strangulation	Also called choking or suffocating, is when a person has survived “having pressure applied over the neck by any means” obstructing the airflow and blood vessels in the neck region. Pressure may be applied by one or two hands, forearm (as in a chokehold), other body parts or ligature. Can occur once or many times by different means. Each time is dangerous.

3. Guideline

[SARC](#) can be contacted via King Edward Memorial Hospital switchboard (08) 6458 2222 (24hours/day) for Nursing, Midwifery or Medical staff advice.

3.1. SARC role

- SARC Doctors and Counsellors are available for advice.
- Triage referrals for patients presenting after a recent sexual assault.

- Refers patient to ED if an injury, acute psychiatric illness, and/or intoxication is suspected and cannot be safely managed at SARC.
- SARC Doctor and Counsellor may attend the referring hospital to see the patient if they require continuing ED or hospital treatment.
- SARC Counsellor available to explain the benefits of the SARC service directly to patient if required.
- Will see patients regardless of whether they intend to report to the WA Police.

3.2. Patient considerations

- When assessing patients, consideration will be given to the patient's gender, ethnicity, religious needs, age and developmental stage in line with trauma informed care principles.
- In accordance with the [SMHS Chaperone policy](#) a chaperone must be present during the patient's assessment and treatment unless refused by the patient as per section 2.1 of the policy
- Patients disclosing sexual assault in clinical areas other than ED are to be assessed against criteria in 3.3:
 - Patients who answer yes to any criterion are to be referred to ED.
 - Patients who answer no to all criteria are to be advised of the SARC service and referred if they are agreeable.

3.3. Criteria for referral to ED

- Refer to ED if the patient:
 - has sustained any injuries
 - is in pain
 - received a blow to the head
 - has reported a loss of consciousness
 - has any vaginal or anal bleeding
 - was or could have been pregnant prior to the incident
 - has history of alcohol and/or drug use associated with the incident
 - either currently or recently suffered any significant mental health problems
 - had any pressure applied to the neck (i.e. non-fatal strangulation)
- Where the answers are NO to the above, contact [SARC](#) for direct patient referral.
- Refer to the [SARC Emergency Care: History & Checklist](#) and document findings in the patient's medical record (MR).

3.4. Timing of the assault

- If the assault was more than 2 weeks ago, and medical issues have been addressed, provide the patient with SARC counselling information and contact Informed consent
- SARC Doctors may only examine and collect forensic evidence from a patient who has capacity and who can give informed consent (refer to updated WA Health [Consent to Treatment Policy](#) (2023) and the [WA Clinician Consent to Treatment Flowchart](#).)
- If the patient is under significant influence of drugs and/or alcohol notify the SARC Doctor. A forensic examination may be delayed until the person has capacity.
- If there is evidence of impaired mental health, a psychiatric opinion is required prior to examination.
- If intellectually impaired or unconscious, and the WA Police are involved, a forensic examination can be performed via the [Criminal Investigation Act 2006](#). If the WA Police are not involved, contact the SARC Doctor to discuss the consent process.
- If the patient is less than 18 years old, discuss with the SARC Doctor.

3.5. Assessment

- Use the First Response Pack 'Sexual Assault Checklist' to guide history taking and management.
- Specifically ask the patient about vaginal, oral and anal penetration, and any objects used.
- Specifically ask patients presenting after physical assault in the context of family and domestic violence, about sexual assault
- Specifically ask about non-fatal strangulation
- Clearly document all findings in the patient's medical record
- Assess injuries specific to penetration, such as vaginal or anal pain or bleeding, and abdominal pain. If serious injuries are suspected, consider the need for surgical or gynaecological referral. Such referrals and any internal examinations should be in consultation with the SARC Doctor so that arrangements for an appropriate forensic examination can be made (refer to Section 3.7 Early forensic specimens).
- Assess for complications of non-fatal strangulation. Refer to the Women and Newborn Health Service guide on non fatal strangulation.
- Assess and treat intoxication, poisoning, injuries and psychiatric emergencies as per standard management protocols.

- Clearly document all findings in the patient's medical record and/or consider using the Trauma proforma.

3.6. Management

- Consider:
 - Possibility of current pregnancy.
 - Past or current mental health history.
 - History of violence or aggression by the patient.
 - Patient safety (from self, others, and/or a perpetrator)
 - A referral to Social work should be offered to all patients to assess safety and is mandatory if patient is less than 18 years old.

If a patient is less than 18 years old, mandatory reporting of child sexual abuse is legally required (refer to the [CHILDREN AND COMMUNITY SERVICES AMENDMENT BILL 2021](#))

- Once medically stable/treated, and if the patient is agreeable, refer them to SARC by calling the KEMH switchboard.
 - Email: SARCDrs@health.wa.gov.au, or Fax (08)-9381-5426 SARC a copy of the medical notes, nursing notes, any medication given, and a discharge summary.
- If the patient is not agreeable to a referral to SARC, provide them with information about SARC resources. and document this in the medical record.

3.7. Early forensic specimens

- Early collection of forensic specimens may be performed in ED to allow the patient to eat, drink and void, without loss of evidence.
- Early Evidence Kit (EEKs) specimens are either self-collected by the patient or collected by the healthcare worker, depending on the type of specimen. Each EEK contains detailed instructions on how to collect each specimen.
- Forensic Toxicology Kits are available in ED and used if a drug facilitated assault is suspected. Each Kit contains detailed instructions on how to collect each specimen
- Educational videos which demonstrate the use of kits and how to collect each type of specimen are available on the SARC Education SharePoint site. The QR code and internet address are available in [Appendix 1](#)

- Refer also to the FSFHG “Forensic Evidence Collection” policy and procedure.
- Contact the SARC Doctor for advice.

3.8. Emergency contraception

- Where the patient declines referral to SARC or is not transferring directly to SARC, emergency contraception should be considered and discussed with the patient:
 - Ulipristal 30mg as a single dose. More effective than Levonorgestrel. Can be given up to 120 hours post unprotected intercourse but is more effective the sooner it is taken.
 - Levonorgestrel 1.5mg as a single dose. Can be taken up to 96 hours after unprotected intercourse but is more effective the sooner it is taken.
 - Exceptionally copper IUD insertion can be considered up to 120-hours following unprotected intercourse.

3.9. Sexually transmitted infections and blood borne virus (BBV) transmission

- Baseline investigations, consider:
 - HIV, Hepatitis B and C, and Syphilis serology
 - First void urine: Chlamydia trachomatis and N. gonorrhoeae PCR
- Discuss with SARC Doctor prior to taking any genital swabs. The following is to be performed **only** if the patient is not being seen by SARC for a forensic examination:
 - External genital examination. The examination and any injuries are to be documented in the patient’s MR
 - High vaginal swab: MC&S (or blind or self-obtained low vaginal swab (SOLVS))
 - Endocervical swab: MC&S and *Chlamydia trachomatis*, *Trichomonas* and *N. gonorrhoeae* PCR (or blind or SOLVS)
 - Throat and/or anal swabs (Chlamydia/Gonorrhoea), if indicated.
- Prophylactic treatment, consider:
 - Azithromycin 1g orally stat
 - Ceftriaxone 500mg IM or IV stat, if high risk gonorrhoea or if the patient is unlikely to attend follow-up.
 - Hepatitis B vaccination.
 - Hepatitis B immunoglobulin, if not known to be immune and a high risk exposure.
 - HIV anti-retroviral therapy. Discuss with Infectious Diseases/SARC Doctor

- HIV non-occupational post exposure prophylaxis(nPEP) starter pack is available in ED containing a 28--day supply.
- eReferral to the FSFHG Sexual Health Clinic for all suspected or proven sexually acquired infections and for follow-up of HIV nPEP.

4.Compliance/performance monitoring

- The ED Head of Service and Nurse Unit Manager are responsible for monitoring compliance with this document. Compliance will be monitored via routine clinical incident review processes.

5. Related legislation

- [Criminal Investigation Act 2006](#)
- [CHILDREN AND COMMUNITY SERVICES AMENDMENT BILL 2021](#)

6. Related policy documents

- WA Health:
 - [WA Health Consent to Treatment policy 2023 \(MP 0175/22\)](#)
 - [Mandatory Reporting of Child Sexual Abuse Training Policy \(MP 0166/21\)](#)
 - [Coordinated medical and forensic and counselling response to patients who experience a recent sexual assault and present to an emergency department \(OP1928\)](#)
- SMHS:
 - [Chaperone \(SMHS:169\)](#)
 - [Standard and transmission-based precautions \(SMHS: 237\)](#)
- FSFHG:
 - Family and Domestic Violence (FSH-HW-PRO-0007)
 - Forensic Evidence Collection (FSH-ED-POL-0002)
 - Standard Precautions (FSFH-IPM-POL-0005)

7.Related standards

- NSQHS Standards:
 - Clinical Governance
 - Partnering with Consumers

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The hard copy of this document may be out of date. To ensure you are reading the current version, check the policy and guideline site on the FSFHG Policy Intranet.

- Comprehensive Care

8. Other related documents

- Women and Newborn Health Service: [Non-Fatal Strangulation \(NFS\) in the context of Intimate Partner Violence: A guide for clinicians](#)
- Sexual Assault Resource Centre patient resource: [Important Information after sexual assault](#)

9. Authorisation

EXECUTIVE SPONSOR: Medical Director - Service 4					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	January 2015	SARC & FSH ED Consultant	Clinical Commissioning	Co-Director Service 4	01/2018
2	10/2018	SARC & FSH ED Consultant	Governance for Safety and Quality in Health Service Organisations	FSFHG Policy Committee	10/2021
3	01/2022	SARC & FSH ED Consultant	NMPGC, DTC	FSFHG Policy Committee	01/2025
4	07/2025	FSH ED Consultant	NMPGC; DTC, ED CGC, SMHS AHST	FSFHG Policy Committee	07/2028

10. Appendices

10.1. Appendix 1 SARC Education Sharepoint page

- The [SARC Education SharePoint page](https://wahealthdept.sharepoint.com/teams/SARCEducationalsite?e=1%3A46a391e6d14a43478c36e4590d5ebea6) location is located at:
<https://wahealthdept.sharepoint.com/teams/SARCEducationalsite?e=1%3A46a391e6d14a43478c36e4590d5ebea6>SARC Education – Home.
- QR Code to Sharepoint page:

